

RAG Example

Question

"What key social and environmental factors have been identified as influencing mental health outcomes, and how do recent studies suggest addressing them?":

Baseline Model

"What key social and environmental factors have been identified as influencing mental health outcomes, and how do recent studies suggest addressing them?": {

"retrieved_chunks": [

"Integral Definition and Conceptual Model of Mental Health.pdf__c0038",

"What is mental health_ Evidence towards a new definition from a mixed methods multidisciplinary international survey.pdf__c0021",

"Integral Definition and Conceptual Model of Mental Health.pdf__c0075",

"What is mental health_ Evidence towards a new definition from a mixed methods multidisciplinary international survey.pdf__c0023",

"What is mental health_ Evidence towards a new definition from a mixed methods multidisciplinary international survey.pdf__c0000",

"Integral Definition and Conceptual Model of Mental Health.pdf__c0011",

"What is good mental health_ A scoping review.pdf__c0012",

"What is good mental health_ A scoping review.pdf__c0013"

],

"retrieved_texts": [

"among\npsychology (26 articles), philosophy (16 articles), sociology (12\narticles), medicine (9 articles), psychiatry (4 articles), biology\n(3 articles), economics (3 articles), education (3 articles), and\nenvironmental sciences (1 article). These contributions come\nmainly from Europe (48%): Netherlands, Finland, Portugal,\nUnited Kingdom, Spain, Greece, Hungary, and Poland; Asia\n(28%): China, Hong Kong, Iran, Bangladesh, and South\nKorea; America (21%): Argentina, Canada, United States,\nMexico, and Suriname; and Africa (3%): South Africa.\nAmong the main mental illnesses considered in empirical\nstudies are depression and anxiety (Kokko et al., 2013 ;\nCharalampopoulou et al., 2020 ;Eidman et al., 2020 ;Fonte\net al., 2020 ;Romero-P\nu00e9rez et al., 2020 ;Syr\nu00e9n et al., 2020 ;\nChan et al., 2021 ;Wan",

"red de \u00b01nition of mental health was the\ngeneral de \u00b01nition of health adapted from Huber et al\n(see table 1).\nAnalysis of the three open-ended items established four\nmajor themes \u00b01Positionality, Social/Environmental Factors,\nParadigms/Theories/Models and the Core Concepts of Mental\nHealth \u00b01and\u00b01ve-directional relationships between them\n(\u00b01gure 1).Positionality represented the overarching per\nspective or point-of-reference from which the Core\nConcepts were derived; whereas Paradigms/Theories/Models\nrepresented the

theoretical framework within which the Core Concepts were described. Core Concepts represented factors related to the individual; these were distinguishable from the Social/Environmental Factors related to society. Five significant relationships between these themes were established,

"health status. With respect to these aspects, biological factors (age, sex, genetics, and special conditions), psychological factors (personality traits, values, motivations, and self-regulation), social factors (educational level, gender, socioeconomic status, marital status, occupation, and family composition), and environmental factors (stressors, discrimination, bullying, challenging, hostile, among others) can be considered in accordance with the contributions of various authors [e.g., DeNeve and Cooper, 1998; Lyubomirsky et al., 2005; Keyes et al., 2010; American Psychiatric Association, 2013; Lim, 2017; World Health Organization (WHO), 2018; among others]. Based on this information, Figure 3 shows the explanatory model of the integral definition of mental health as a result of the Grounded Theory",

"tal factors impacted the core concepts (Direction D) and reciprocally, how the core concepts could influence society (Direction E) (tables 3 and 4). Feedback from the conference focus groups showed support for these bidirectional relationships but questioned whether there was evidence for other direct relationships, specifically the impact of Social/Environmental Factors on both Paradigms/Theories/Models and Positionality . A second round of data analysis confirmed these relationships were not explicitly reported by respondents in the survey. Respondents did not discuss Table 1 Current definitions of mental health and participant rank ordering from most to least preferred Definition of mental health Most preferred (%) Second most preferred (%) \nPublic Health Agency of Canada 18 \n 2013 Mental health",

"What is mental health? Evidence towards a new definition from a mixed methods multidisciplinary international survey \n Laurie A Manwell, 1, 2 Skye P Barbic, 1, 3 Karen Roberts, 1 Zachary Durisko, 1 \n Cheolsoo Lee, 1, 4 Emma Ware, 1 Kwame McKenzie 1 \n To cite: Manwell LA, Barbic SP, Roberts K, et al. \n What is mental health? Evidence towards a new definition from a mixed methods multidisciplinary international survey. BMJ Open 2015; 5:e007079. \n doi:10.1136/bmjopen-2014-007079 \n 25b8 Prepublication history for this paper is available online. \n To view these files please visit the journal online \n (http://dx.doi.org/10.1136/bmjopen-2014-007079). \n Received 4 November 2014 \n Revised 21 January 2015 \n Accepted 22 January 2015 \n For numbered affiliations see end of article. \n Correspondence to \n Dr Laurie A Manwell; \n lauriemanwell@gmail.com",

"orts between natural sciences (biology, neuroscience, medicine, etc.) and social sciences (psychology, sociology, philosophy, economics, etc.) converge to give meaning to mental health. \n With respect to the factors related to mental health, the following aspects can be considered: biological (age, sex, genetics, and special conditions), psychological (personality traits, values, motivations, and aspects of self-regulation), social (educational level, gender, socioeconomic status, marital status, occupation, and family composition), and environmental (stressful, challenging, hostile, among others); \n by the contributions of several authors [for example, DeNeve and Cooper, 1998; Lyubomirsky et al., 2005; Keyes et al., 2010; American Psychiatric Association, 2013; Lim, 2017; World Health Organization",

"eventive \napproaches has somewhat obscured a parallel area of \nknowledge, which pertains to the promotion of good mental \nhealth. Indeed, strengthening of good mental health has \nreceived much less empirical research than the prevention \nof poor mental health and mental disorders. The reasons for this gap of knowledge are not completely clear, however, \nit is possible that this is due to the medical, disease- and \nde\ufb01cit-centred approach that has dominated clinical psy- \nchiatry over the past decades (Arango et al., 2018). Another \nreason is that good mental health is culturally sensitive \n(Snodgrass et al., 2017) and, as such, research attempts \nin this \ufb01eld have been sporadic and idiosyncratic. Further- \nmore, no overall conceptual",

"mental health is culturally sensitive \n(Snodgrass et al., 2017) and, as such, research attempts \nin this \ufb01eld have been sporadic and idiosyncratic. Further- \nmore, no overall conceptual framework for promoting good \nmental health has been validated yet. The resulting picture \nis fragmented and lacking a coherent view. On a pragmatic \nlevel, there is no consensus of what exactly good mental \nhealth is. This situation is particularly problematic because \nit impedes comparative research. For example, although \nthere is wide consensus that it is important to achieve good \nmental health, especially in young people, currently it is \nnot clear what core domains are to be targeted. On the \none hand, there are no validated assessment tools that \ncan"

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"golden_chunks": [

"Social and Environmental Factors reflected the societal factors external to the individual that affect mental health. Although many respondents listed the basic necessities for general health/mental health (eg, housing, food security, access to health services, equitable access to public resources, childcare, education, transportation, support for families, respect for diversity, opportunities for building resilience, selfesteem, personal and social efficacy, growth, meaning and purpose, and sense of safety and belonging, and employment), some also recommended approaches to achieving social equity (eg, \u201cmental health needs to be protected by applying antiracism, antioppression, antidiscrimination lens to prevention and treatment\u201d) (figure 1, Direction D). A distinct category of human rights developed from responses to the third open-ended question (eg, \u201cWhat is missing?\u201d)",

"In contrast, Social and Environmental Factors reflected respondents\u2019 emphasis on factors that are external to the individual and which can influence the core concepts of mental health. Many respondents reiterated the basic necessities for general health/mental health, similar to the foundations of Maslow\u2019s hierarchy of needs, and their recommendations for achieving social equity. Descriptions of the core concepts of mental health were highly influenced by respondents\u2019 Positionality and Paradigms/Theories/Models of reference, which often propelled the discourse of \u201cWhat is mental health?\u201d in opposing directions. The debate as to whether mental health and illness are distinct constructs, or points of reference on a continuum of being, was a common theme. Respondents were either, adamant in asserting the distinction between the descriptive or prescriptive nature of the core concepts, or, ardent in integrating them, producing ideas such as describing mental health as a life free of poverty, discrimination, oppression, human rights violations and war. Respondents\u2019 made repeated references to human rights, suggesting that a basic standard, analogous to a legal definition, is required, and that \u2018a human rights, political,

economic and ecosystem perspective should be included. Again, in the tradition of Hume's brought distinction, several respondents cautioned that problems of living, such as poverty, vices and social injustices should not be defined as medical problems. The significance of this issue cannot be understated: while we asked respondents what the core concepts of mental health are, overwhelmingly they answered in terms of what they should be. This finding is similar to other issues in public health policy that address instances of conflating scientific evidence with moral argument.",

"With respect to the factors related to mental health, the following aspects can be considered: biological (age, sex, genetics, and special conditions), psychological (personality traits, values, motivations, and aspects of self-regulation), social (educational level, gender, socioeconomic status, marital status, occupation, and family composition), and environmental (stressful, challenging, hostile, among others); by the contributions of several authors [for example, DeNeve and Cooper, 1998; Lyubomirsky et al., 2005; Keyes et al., 2010; American Psychiatric Association, 2013; Lim, 2017; World Health Organization (WHO), 2018; among others]. In relation to mental health care options, World Health Organization (WHO) (2009) made a distinction between informal services (self-care and community care), and formal services (primary care, community services, psychiatric in general hospitals, long-stay facilities, and specialized services). However, it emphasized that self-care is essential and occurs simultaneously with other services, as the person self-manages, without the intervention of a professional, their mental health problems; in addition to promoting and fostering their recovery, and better mental health. Based on the above, it is clear that mental health is a complex concept, which, from different approaches, has given rise to a proliferation of conceptual notions, making it difficult to measure and create strategies for improvement. In this sense, it is necessary to reduce the gap by analyzing the input toward a proposal for a unifying concept. Therefore, the aim of the present study was to analyze different conceptual contributions regarding mental health through the Systematic Literature Review (SLR). Literature reviews related to the definition of mental health are few, mostly focused on analyzing specific related aspects, for example, mental health in specific populations (Suhaiban et al., 2019), the type of care people receive (Bakker et al., 2016), related factors (Shalaby and Agyapong, 2020), among others. The concept of mental health was identified by the work of Muñoz et al. (2016), which was based on an analysis of positive mental health, and the work done by Wang and Lai (2022), in which they used two paradigms as references in their analysis: the absence of disease and positive mental health. As can be seen, the literature reviews on mental health are diverse and polysemic.",

"These are aspects related to the person that has an association with one's mental health status. With respect to these aspects, biological factors (age, sex, genetics, and special conditions), psychological factors (personality traits, values, motivations, and self-regulation), social factors (educational level, gender, socioeconomic status, marital status, occupation, and family composition), and environmental factors (stressors, discrimination, bullying, challenging, hostile, among others) can be considered in accordance with the contributions of various authors [e.g., DeNeve and Cooper, 1998; Lyubomirsky et al., 2005; Keyes et al., 2010; American Psychiatric Association, 2013; Lim, 2017; World Health Organization (WHO), 2018; among others]. Based on this information, Figure 3 shows the explanatory model of the integral

definition of mental health as a result of the Grounded Theory approach (Strauss and Corbin, 1990). The present definition and its respective explanatory model bring value to the field of mental health because it is a concept that integrates contributions from various disciplines such as psychology, sociology, psychiatry, philosophy and education; unites diverse paradigms such as positive mental health, mental health based on the absence of illness and mental health based on a state of balance; focuses on self-care, recognized by World Health Organization (WHO) (2009) as an essential element of care for mental health; seeks to empower the person to manage one's mental health problems; is person-centered by considering the person's values and motivations; is based on intrapersonal and interpersonal aspects, two distinct dimensions that converge to give meaning to mental health; is relative to the person by taking into account various factors that vary according to development, context and personal characteristics; and prevents stigmatization by focusing the concept on an internal process of personal balance, rather than a state of complete wellbeing and absence of mental disorder that could be utopian. The present research is an approach that seeks to understand the concept of mental health from different disciplines; however, it is recognized that the keywords could exclude other disciplines that contribute to its understanding; therefore, it is recommended to analyze the concept from other disciplines not considered in the research. In addition, the complexity of the concept and its associated factors is recognized; therefore, it is suggested that future research must go deep into the study of the existing relationships between the elements that conform to the proposed explanatory model of mental health in different contexts. The original contributions presented in the study are included in the article/Supplementary material, further inquiries can be directed to the corresponding author. MC-S conducted a systematic review of mental health paradigms. JR-M provided a context in mental health's different paradigms. The authors worked together to edit and integrate contents in the manuscript. All authors contributed to the article and approved the submitted version."

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Quantitative Evaluation

Retrieval Metrics & Ranking Scores

- **Precision:** 0.38
- **Recall:** 0.75
- **F1 Score:** 0.50
- **Coverage:** 0.65
- **Mean Max Similarity:** 0.60

Ranking Metrics

- **MRR (Mean Reciprocal Rank):** 0.50
- **nDCG (Normalized Discounted Cumulative Gain):** 1.00

Qualitative Evaluation

Scores:

- **Completeness Score:** 3 / 5
- **Grounding Score:** 4 / 5
- **Relevance Score:** 4 / 5

Missing Key Points:

1. Specific recent studies addressing social and environmental factors
2. Concrete methods for addressing these factors

High-Level Rationale:

The chunks provide some information on the factors influencing mental health outcomes, such as social and environmental factors, but lack specific recent studies and concrete methods for addressing them, as seen in chunk_id=Integral Definition and Conceptual Model of Mental Health.pdf__c0075 and chunk_id=What is mental health_ Evidence towards a new definition from a mixed methods multidisciplinary international survey.pdf__c0021. The chunks are relevant to the question but do not fully answer it, resulting in a moderate completeness score and a moderate to high grounding score.

Best-performing Model (Paragraph-based chunking with Reranking)

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{
  "retrieved_chunks": [
    "What is mental health_ Evidence towards a new definition from a mixed methods multidisciplinary international survey.pdf__c0013",
    "What is mental health_ Evidence towards a new definition from a mixed methods multidisciplinary international survey.pdf__c0015",
    "Integral Definition and Conceptual Model of Mental Health.pdf__c0002",
    "Integral Definition and Conceptual Model of Mental Health.pdf__c0023",
    "Correlation_between_resilience_and_mental_health.txt__c0017",
    "What is mental health_ Evidence towards a new definition from a mixed methods multidisciplinary international survey.pdf__c0008"
  ],
  "retrieved_texts": [
    "doi:10.1136/bmjopen-2014-007079Open Access\nvalues and beliefs in moving towards the full implementation of none of the 2019s goals and ambitions ( figure 1 ). Social and Environmental Factors reflected the societal factors external to the individual that affect mental health. Although many respondents listed the basic necessities for general health/mental health (eg, housing, food security, access to health services, equitable access to public resources, childcare, Figure 2 Positionality. The overarching perspective or point-of-reference used to describe the constructs of mental health and illness. Figure 3 Complexity. Descriptions of mental health in relation to hierarchical levels, and/or spatial directions, and/or temporal trajectories. Manwell LA, et al.BMJ Open 2015; 5:e007079. doi:10.1136/bmjopen-2014-007079 7Open Access\neducation, transportation, support for families, respect for diversity, opportunities for building resilience, self-esteem, personal and social efficacy, growth, meaning and purpose, and sense of safety and belonging, and employment), some also recommended approaches to achieving social equity (eg, mental health needs to be protected by applying antiracism, antioppression, antidiscrimination lens to prevention and treatment (figure 1, Direction D). A distinct category of human rights developed from responses to the third open-ended question (eg, What is missing? (figure 1 ).",
    "In contrast, Social and Environmental Factors reflected respondents emphasis on factors that are external to the individual and which can influence the core concepts of mental health. Many respondents reiterated the basic necessities for general health/mental health, similar to the foundations of Maslow's hierarchy of needs, and their recommendations for achieving social equity. Descriptions of the core concepts of mental health were highly influenced by respondents Positionality and Paradigms/Theories/Models of reference, which often propelled the discourse of What is mental health? in opposing directions. The debate as to whether mental health and illness are distinct constructs, or points of reference on a continuum of being, was a common theme. Respondents were either, adamant in asserting the distinction between the descriptive or prescriptive nature of the core concepts, or, ardent in integrating them, producing ideas such as describing mental health as a life free of poverty,
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discrimination, oppression, human rights violations and war. Respondents made repeated references to human rights, suggesting that a basic standard, analogous to a legal definition, is required, and that human rights, political, economic and ecosystem perspectives should be included. Again, in the tradition of Hume's thought is a distinction, several respondents cautioned that problems of living, such as poverty, vices and social injustices should not be defined as medical problems. The significance of this issue cannot be understated: while we asked respondents what the core concepts of mental health are, overwhelmingly they answered in terms of what they should be. This finding is similar to other issues in public health policy that address instances of conflicting scientific evidence with moral argument.

"With respect to the factors related to mental health, the following aspects can be considered: biological (age, sex, genetics, and special conditions), psychological (personality traits, values, motivations, and aspects of self-regulation), social (educational level, gender, socioeconomic status, marital status, occupation, and family composition), and environmental (stressful, challenging, hostile, among others); by the contributions of several authors [for example, DeNeve and Cooper, 1998; Lyubomirsky et al., 2005; Keyes et al., 2010; American Psychiatric Association, 2013; Lim, 2017; World Health Organization (WHO), 2018; among others]. In relation to mental health care options, World Health Organization (WHO) (2009) made a distinction between informal services (self-care and community care), and formal services (primary care, community services, psychiatric in general hospitals, long-stay facilities, and specialized services). However, it emphasized that self-care is essential and occurs simultaneously with other services, as the person self-manages, without the intervention of a professional, their mental health problems; in addition to promoting and fostering their recovery, and better mental health. Based on the above, it is clear that mental health is a complex concept, which, from different approaches, has given rise to a proliferation of conceptual notions, making it difficult to measure and create strategies for improvement. In this sense, it is necessary to reduce the gap by analyzing the input toward a proposal for a unifying concept. Therefore, the aim of the present study was to analyze different conceptual contributions regarding mental health through the Systematic Literature Review (SLR). Literature reviews related to the definition of mental health are few, mostly focused on analyzing specific related aspects, for example, mental health in specific populations (Suhaiban et al., 2019), the type of care people receive (Bakker et al., 2016), related factors (Shalaby and Agyapong, 2020), among others. The concept of mental health was identified by the work of Muñoz et al. (2016), which was based on an analysis of positive mental health, and the work done by Wang and Lai (2022), in which they used two paradigms as references in their analysis: the absence of disease and positive mental health. As can be seen, the literature reviews on mental health are diverse and polysemic."

"Factors influencing mental health These are aspects related to the person that has an association with one's mental health status. With respect to these aspects, biological factors (age, sex, genetics, and special conditions), psychological factors (personality traits, values, motivations, and self-regulation), social factors (educational level, gender, socioeconomic status, marital status, occupation, and family composition), and

environmental factors

(stressors, discrimination, bullying, challenging, hostile, among others) can be considered in accordance with the contributions of various authors [e.g., DeNeve and Cooper, 1998; Lyubomirsky et al., 2005; Keyes et al., 2010; American Psychiatric Association, 2013; Lim, 2017; World Health Organization (WHO), 2018; among others]. Based on this information, Figure 3 shows the explanatory model of the integral definition of mental health as a result of the Grounded Theory approach (Strauss and Corbin, 1990). Conclusion The present definition and its respective explanatory model bring value to the field of mental health because it is a concept that (1) integrates contributions from various disciplines such as psychology, sociology, psychiatry, philosophy and education; (2) unites diverse paradigms such as positive mental health, mental health based on the absence of illness and mental health based on a state of balance; (3) focuses on self-care, recognized by World Health Organization (WHO) (2009) as an essential element of care for mental health; (4) seeks to empower the person to manage one's mental health problems; (5) is person-centered by considering the person's values and motivations; (6) is based on intrapersonal and interpersonal aspects, two distinct dimensions that converge to give meaning to mental health; (7) is relative to the person by taking into account various factors that vary according to development, context and personal characteristics; and (8) prevents stigmatization by focusing the concept on an internal process of personal balance, rather than a state of incomplete wellbeing and absence of mental disorder that could be utopian. The present research is an approach that seeks to understand the concept of mental health from different disciplines; however, it is recognized that the keywords could exclude other disciplines that contribute to its understanding; therefore, it is recommended to analyze the concept from other disciplines not considered in the research. In addition, the complexity of the concept and its associated factors is recognized; therefore, it is suggested that future research must go deep into the study of the existing relationships between the elements that conform to the proposed explanatory model of mental health in different contexts. Data availability statement The original contributions presented in the study are included in the article/ Supplementary material, further inquiries can be directed to the corresponding author. Author contributions MC-S conducted a systematic review of mental health paradigms. JR-M provided a context in mental health's different paradigms. The authors worked together to edit and integrate contents in the manuscript. All authors contributed to the article and approved the submitted version."

"Although the exact mechanisms that resilience promotes mental health remain complicated (91), resilience theory provides a well-established framework to explain its effects. Resilience facilitates mental health by enabling individuals to counteract the negative consequences of exposure to risk factors. This process involves leveraging environmental, FIGURE 6 Sensitivity analysis of the correlation between resilience and negative indicators of mental health. FIGURE 7 Sensitivity analysis of the correlation between resilience and positive indicators of mental health. Luo et al.

10.3389/fpsy.2025.1536553 Frontiers in Psychiatry 10 frontiersin.org social, and personal protective factors to interrupt the progression from risk exposure to pathological outcomes (33). In specific, through the mobilization of these internal and external protective factors,

individuals are able to face up to adverse life events and learn from their struggles to achieve further growth and positive cognitive development (92). As a result, individuals might avoid psychological dysfunction (31), be less likely to suffer from detrimental mental health outcomes (93), and raise the positive indicators such as the perceived well-being (32, 94) to contribute to the promotion of overall mental health (95-97). Additionally, similar positive effects have been explored through theoretical mechanisms. For instance, resilience can promote post-traumatic growth through the mediating effects of internal factors, such as positive emotions, and external factors, such as social support (98).",

"First, respondents' theoretical framework (Direction A) influenced the overarching point-of-reference they used to describe the core concepts and vice versa (Direction B). Positionality and Paradigms/Theories/Models significantly influenced the core concepts respondents provided and the corresponding descriptions (Direction C). Respondents described how social and environmental factors impacted the core concepts (Direction D) and reciprocally, how the core concepts could influence society (Direction E) (tables 3 and 4). Feedback from the conference focus groups showed support for these five-directional relationships but questioned whether there was evidence for other direct relationships, specifically the impact of Social/Environmental Factors on both Paradigms/Theories/Models and Positionality . A second round of data analysis confirmed these relationships were not explicitly reported by respondents in the survey. Respondents did not discuss

Table 1 Current definitions of mental health and participant rank ordering from most to least preferred	Definition of mental health	Most preferred (%)	Second most preferred (%)
Public Health Agency of Canada	Mental health is the capacity of each and all of us to feel, think, and act in ways that enhance our ability to enjoy life and deal with the challenges we face. It is a positive sense of emotional and spiritual well-being that respects the importance of culture, equity, social justice, interconnections and personal dignity	18%	20%
WHO	Mental health is defined as a state of well-being in which every individual realizes his or her own potential, can cope with the normal stresses of life, can work productively and fruitfully, and is able to make a contribution to her or his community	24%	20%
McKenzie K (personal communication, 2014)	A mentally healthy community offers people the ability to thrive. It is one in which people feel a sense of connectedness and there are also networks which link people from all walks of life to each other. There is a strong community identity but despite this the community is welcoming of diversity. People participate in their community, organize to combat common threats and offer support and aid for those in need	14%	28%
Huber et al	Mental health is the ability to adapt and self-manage	15%	28%
None of the existing definitions are satisfactory		30%	68%

Manwell LA, et al. BMJ Open 2015; 5:e007079."

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"golden_chunks": [

"Social and Environmental Factors reflected the societal factors external to the individual that affect mental health. Although many respondents listed the basic necessities for general health/mental health (eg, housing, food security, access to health services, equitable access to public resources, childcare, education, transportation, support for families, respect for diversity, opportunities for building resilience, self-esteem, personal and social efficacy, growth, meaning and purpose, and sense of safety and belonging, and employment), some also recommended

approaches to achieving social equity (eg, \u201cmental health needs to be protected by applying antiracism, antioppression, antidiscrimination lens to prevention and treatment\u201d) (figure 1, Direction D). A distinct category of human rights developed from responses to the third open-ended question (eg, \u201cWhat is missing?\u201d)",

"In contrast, Social and Environmental Factors reflected respondents\u2019 emphasis on factors that are external to the individual and which can influence the core concepts of mental health. Many respondents reiterated the basic necessities for general health/mental health, similar to the foundations of Maslow\u2019s hierarchy of needs, and their recommendations for achieving social equity. Descriptions of the core concepts of mental health were highly influenced by respondents\u2019 Positionality and Paradigms/Theories/Models of reference, which often propelled the discourse of \u201cWhat is mental health?\u201d in opposing directions. The debate as to whether mental health and illness are distinct constructs, or points of reference on a continuum of being, was a common theme. Respondents were either, adamant in asserting the distinction between the descriptive or prescriptive nature of the core concepts, or, ardent in integrating them, producing ideas such as describing mental health as a life free of poverty, discrimination, oppression, human rights violations and war.

Respondents\u2019 made repeated references to human rights, suggesting that a basic standard, analogous to a legal definition, is required, and that \u2018a human rights, political, economic and ecosystem perspective\u2019 should be included. Again, in the tradition of Hume\u2019s \u2018ought\u2013is\u2019 distinction, several respondents cautioned that problems of living, such as \u2018poverty, vices and social injustices\u2019 should not be defined \u2018as medical problems\u2019. The significance of this issue cannot be understated: while we asked respondents what the core concepts of mental health are, overwhelmingly they answered in terms of what they should be. This finding is similar to other issues in public health policy that address instances of \u2018conflating scientific evidence with moral argument\u2019.",

"With respect to the factors related to mental health, the following aspects can be considered: biological (age, sex, genetics, and special conditions), psychological (personality traits, values, motivations, and aspects of self-regulation), social (educational level, gender, socioeconomic status, marital status, occupation, and family composition), and environmental (stressful, challenging, hostile, among others); by the contributions of several authors [for example, DeNeve and Cooper, 1998; Lyubomirsky et al., 2005; Keyes et al., 2010; American Psychiatric Association, 2013; Lim, 2017; World Health Organization (WHO), 2018; among others]. In relation to mental health care options, World Health Organization (WHO) (2009) made a distinction between informal services (self-care and community care), and formal services (primary care, community services, psychiatric in general hospitals, long-stay facilities, and specialized services). However, it emphasized that self-care is essential and occurs simultaneously with other services, as the person self-manages, without the intervention of a professional, their mental health problems; in addition to promoting and fostering their recovery, and better mental health. Based on the above, it is clear that mental health is a complex concept, which, from different approaches, has given rise to a proliferation of conceptual notions, making it difficult to measure and create strategies for improvement. In this sense, it is necessary to reduce the gap by analyzing the input toward a proposal for a unifying concept. Therefore, the aim of the present study was to analyze different conceptual contributions

regarding mental health through the Systematic Literature Review (SLR). Literature reviews related to the definition of mental health are few, mostly focused on analyzing specific related aspects, for example, mental health in specific populations (Suhaiban et al., 2019), the type of care people receive (Bakker et al., 2016), related factors (Shalaby and Agyapong, 2020), among others. The concept of mental health was identified by the work of Mu\u00f1oz et al. (2016), which was based on an analysis of positive mental health, and the work done by Wang and Lai (2022), in which they used two paradigms as references in their analysis: the absence of disease and positive mental health. As can be seen, the literature reviews on mental health are diverse and polysemic."

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Quantitative Evaluation

Metrics & Ranking Scores

- **Precision:** 0.83
- **Recall:** 1.00
- **F1:** 0.91
- **Coverage:** 0.84
- **Mean Max Similarity:** 0.75

Ranking Metrics

- **MRR (Mean Reciprocal Rank):** 1.00
- **nDCG (Normalized Discounted Cumulative Gain):** 1.00

Qualitative Evaluation

Evaluation Summary

- **Completeness Score:** 3 / 5
- **Grounding Score:** 4 / 5
- **Relevance Score:** 5 / 5

Missing Key Points:

1. Specific recent studies that suggest addressing social and environmental factors
2. Concrete methods for addressing these factors

High-Level Rationale:

The chunks provide a comprehensive overview of the social and environmental factors that influence mental health outcomes, including factors such as housing, food security, access to health services, and respect for diversity. However, they do not provide specific recent studies that suggest addressing these factors, and the chunks lack concrete methods for addressing them, as seen in chunks [1] and [3]. The information is well-grounded in the chunks, but the answer is not fully complete due to the lack of specific recent studies and concrete methods.